INDIVIDUAL CASE SAFETY REPORT (ICSR)

Pharmacovigilance Assessment | Prepared by a Drug Safety Associate (simulated)

Report type	Spontaneous, initial report
Date of report	12-11-2026
Suspected drug	Amoxicillin
Primary event	Skin rash (with itching, nausea, dizziness)
Final assessment	Non-serious | Expected | Causality: Probable

01. ADR Identification

A 45-year-old female developed rash, itching, nausea and dizziness after starting amoxicillin. Review of symptoms, medication history and timeline:

- Amoxicillin was started on Day -5 and symptoms began on Day -2, about 3 days after starting the drug.
- Rash and itching are typical hypersensitivity reactions to penicillins; nausea and dizziness are also listed effects of amoxicillin.
- The only other medicine is paracetamol (as needed), which rarely causes rash at normal doses, so it is the less likely cause.
- **Conclusion:** the reaction is drug-related and the **suspected drug is amoxicillin**.

02. Reporting (ICSR details)

A. Patient information

Patient initials	XX
Age / Gender	45 years / Female
Weight / Height	Not specified

B. Suspected medicine

Medicine	Amoxicillin (penicillin antibiotic)
Dose / Frequency	Not specified in case
Route	Oral
Start date	Day -5
Stop date	On reporting (Day 0)
Concomitant medicine	Paracetamol, as needed

C. Adverse reaction

Reaction / Event	Skin rash, itching, nausea, dizziness
Date of onset	Day -2 (3 days after starting the drug)
Dechallenge	Positive: reaction improved after stopping amoxicillin
Rechallenge	Not done

D. Outcome

Outcome	Recovered after stopping the drug

Action taken	Amoxicillin withdrawn; non-penicillin antibiotic advised; penicillin allergy to be recorded

E. Reporter

Name / Role	Sakshi, Drug Safety Associate (assignment simulation)

Case timeline

Day	Event
Day -5	Amoxicillin started
Day -2	Rash, itching, nausea and dizziness began
Day 0	Reaction reported; amoxicillin stopped
After stopping	Symptoms recovered (positive dechallenge)

03. Causality Assessment

Criterion	Finding	Result
Temporal relationship	Onset 3 days after starting amoxicillin; plausible	Yes
Dechallenge	Symptoms recovered after stopping the drug	Positive
Alternative causes	No other cause identified; paracetamol unlikely	None found
Rechallenge	Not done (not advisable after an allergic reaction)	Not done

WHO-UMC: PROBABLE. It is not 'Certain' because rechallenge was not performed. **Naranjo score = 6** (previous reports +1, event after drug +2, improved on stopping +1, no alternative cause +2), which falls in the Probable range (5 to 8). Both methods agree.

04. Classification

Parameter	Classification	Reason
Seriousness	**Non-serious**	No death, life threat, hospitalization, disability or other medically important condition
Expectedness	**Expected**	Rash, itching, nausea and dizziness are listed in the known safety information of amoxicillin
Clinical impact	Low to moderate	Mild and reversible after stopping the drug

As the case is non-serious and expected, routine reporting is sufficient; no expedited reporting is needed.

Conclusion and recommendations

- Suspected drug: **amoxicillin**; causality **Probable**; **non-serious** and **expected**.
- Stop amoxicillin and use a non-penicillin antibiotic.
- Record penicillin allergy in the patient's file and counsel the patient.
- Submit the report to PvPI (Pharmacovigilance Programme of India) for routine processing.